

Public Health Emergency Operations Center
"COUSP DRC"

MVE-17 Incident Management System

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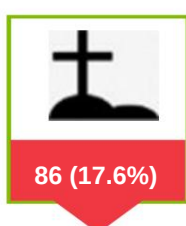
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°22/MVB_05/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	- Ituri (17/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara - North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha - South Kivu (1/34): Miti-Murhesa
Reporting date	June 5, 2026
Publication date	June 6, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



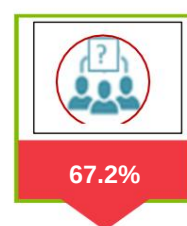
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data currently being harmonized

1. HIGHLIGHTS

- **Thirty-six (36) new confirmed cases, including 4 deaths**, were reported as of June 5, 2026. in the health zones of **Bunia (15), Mongbwalu (10), Rwampara (7), Nyankunde (2), Kilo (1), Lita (1)**.
- **A confirmed patient** of Ebola virus disease has been declared cured (Rwampara Health Zone General Referral Hospital).
- Identification of **11 confirmed cases** in 3 private healthcare facilities. The process of Referral to standardized care structures (CTE) is in progress.
- Start of assessments in healthcare facilities in the Mongbwalu area.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

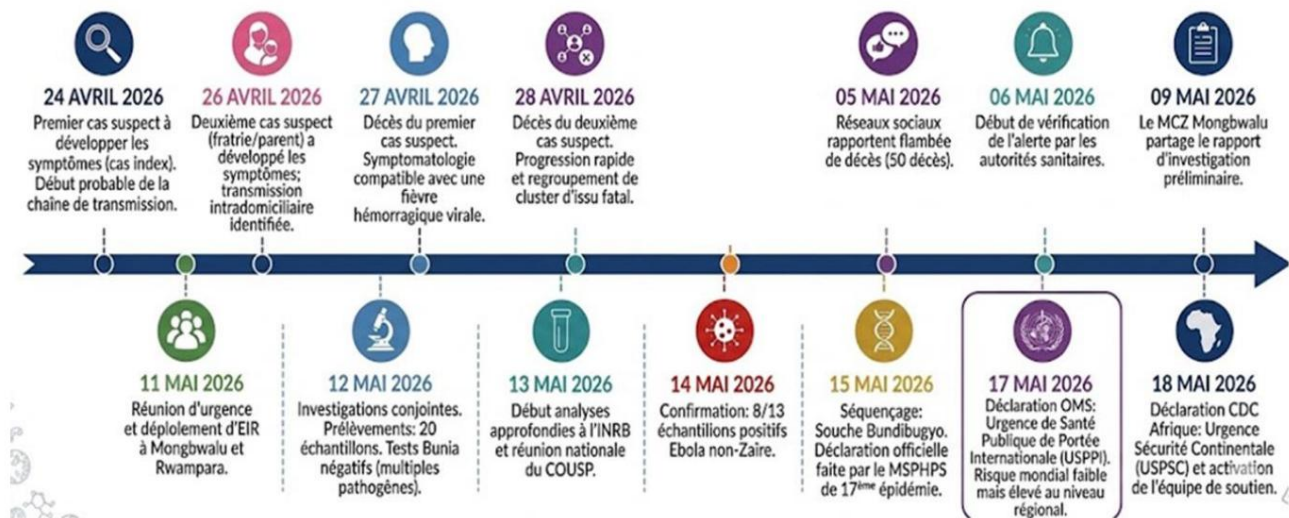


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 5, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24 hours)
Ituri	460	70	15.2%	17 out of 36 (47.2%)	36
North Kivu	25	15	60.0%	7 out of 34 (20.6%)	0
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	488	86	17.6%	25 out of 104 (24.0%)	36

No new health zones have been affected in the last 48 hours. Of the three affected provinces, Ituri remains the national epicenter with 94.2% of confirmed cases. The overall case fatality rate in the provinces

The percentage of affected areas remains low, particularly in Ituri (15.2%), while it is high in North Kivu province (60.0%). South Kivu is the least affected, with its last confirmed case dating back to May 26, 2026.

- North Kivu has a worrying case fatality rate of 60.0%, which can be explained by delays in care and escape of three confirmed cases from care facilities.

3.2 Distribution by health zone (Cumulative as of 05/06/2026)

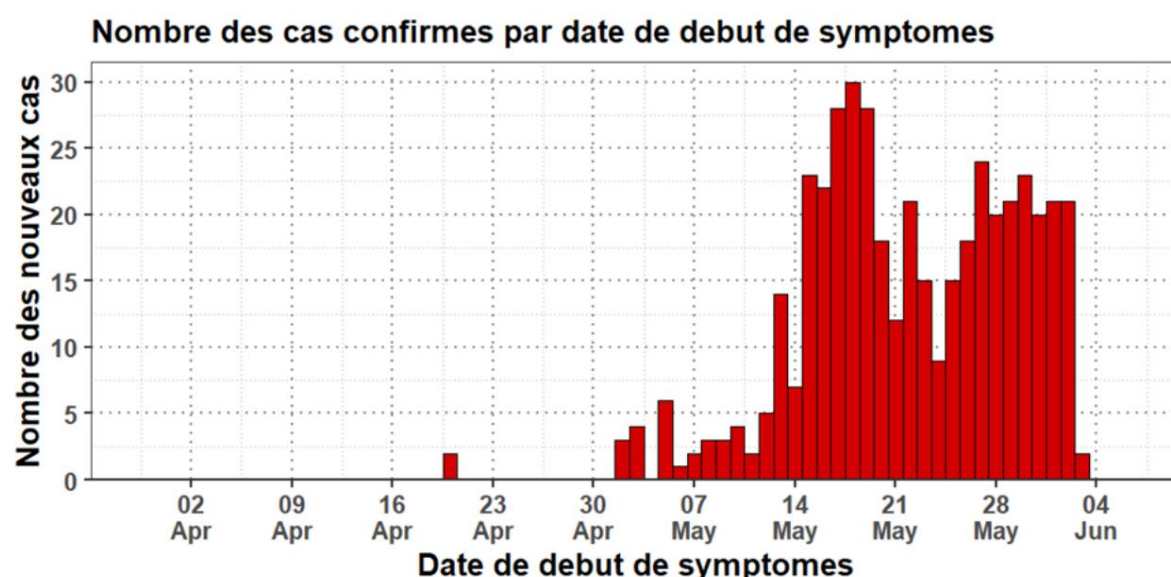
- **Ituri Province (460 cases):** Bunia (138), Rwampara (93), Mongbwalu (74), Nyankunde (24), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damas (3), Kilo (4), Nizi (5), Mangala (1), Aungba (1), Gety (1), Komanda (3), Lita (4), Logo (2). Ninety-four (94) additional cases are being ventilated.
- **North Kivu Province (25 cases):** Katwa (11), Beni (5), Butembo (4), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time analysis (*missing data from June 5, 2026*) : A significant number of confirmed cases

(cohort 1) was symptomatic in the period between May 14 and May 23, 2026, suggesting increased contamination from a probable common outbreak, with a peak observed on May 18, 2026.

Subsequently, a number of confirmed cases (cohort 2) began showing symptoms between May 25th and June 3rd.

demonstrating the spread of the disease and potentially constituting a significant reservoir. An increase in cases may be recorded if appropriate measures are not implemented very quickly.



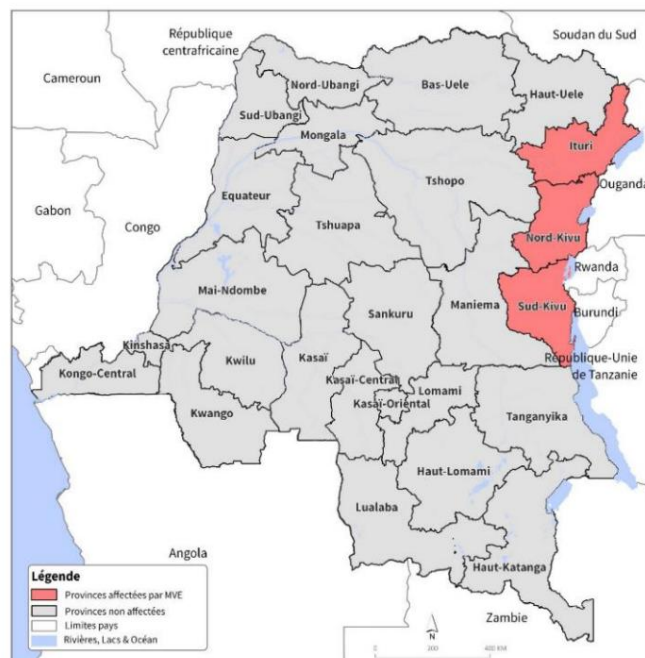
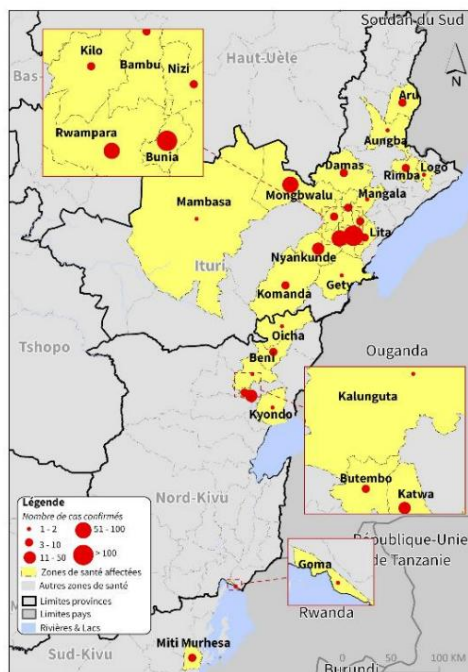
Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 447.

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 5, 2026

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI			
Bunia	138	14	10.1%
Rwampara	93	19	20.4%
Mongbwalu	74	21	28.4%
Nyankunde	24		4.2%
Bamboo	5	1	40.0%
Aru	3		33.3%
Kilo	4		25.0%
Nizi	5	2	0.0%
Mangala		1	0.0%
Damascus	1	1	0.0%
Aungba		0	0.0%
Gety		0	0.0%
Komanda	3	0	0.0%
Lita	1	0	0.0%
Logo	1 3 4 2	0 0 0 0	0.0%

Mambasa	2		50.0%
Rimba	3	10	0.0%
Other ZS (undisclosed data)		10	10.6%
Ituri subtotal	94,460	70	15.2%
NORTH KIVU			
Katwa	11	7	63.6%
Blessed	5	3	60.0%
Butembo	4	2	50.0%
Oicha	2	2	100.0%
Kalunguta			100.0%
Kyondo		1	0.0%
Goma	111	00	0.0%
North Kivu subtotal	25	15	60.0%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	488	86	17.6%

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 5, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- **National / Ituri:**
 - Debriefing of the coordination team and the various technical and financial partners on the mission of the Nizi ZS, recently assigned.
 - Organizing a press conference, the Director General of the INSP spoke with the local press around the positive management of rumors, adherence to the EDS practice, referral of those exhibiting symptoms of the disease to healthcare facilities, importance of a CTE
 - **Deployment of a joint mission in the Mongbwalu health zone to assess the establishments of care on the implementation of continuity of care.**
- **North Kivu:** The Incident Management System (IMS) meeting was held at the Provincial Health Division (DPS) to review the evolution of the epidemic and operational priorities.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 5, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	97	340	8	445
Alerts investigated	97	340	8	445
Investigation rate	100.0%	100.0%	100.0%	100.0%
Alerts confirmed	72 (74.2%)	40 (11.8%)	7 (87.5%)	119

At the conclusion of investigations conducted on June 5, 2026, 445 alerts were received, all of which were investigated. As a result, **119 suspected cases were recorded** in the three affected provinces, of which 60.5% were located in Ituri province, 33.6% in North Kivu and 5.8% in South Kivu. However, the largest number of alerts (n=340) came from North Kivu, but with a low rate of validated alerts (11.8%).

Table 3. Contact tracing of confirmed cases as of June 5, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	4,281	2,701	63.1%
North Kivu	689	578	83.9%
South Kivu	187	184	98.4%
Total	5,157	3,463	67.2%

- Routine field surveillance activities (reporting alerts, investigations, active searching, listing and monitoring contacts, retrospective review of cases and deaths, etc.)
- In-depth investigations around confirmed cases in the affected health zones (Bunia, Nyankunde and Rwampara).

Compared to the previous 24 hours, contact tracing has improved in Ituri province (54.6% vs. 63.1%) and in North Kivu province (72.0% vs. 83.9%). However, contact tracing efforts will need to be strengthened to achieve optimal coverage (at least 90%) in the aforementioned provinces.

Update on checkpoint and point of entry (PoC/PoE) activities

- Deployment of a national PoE/PoC supervisor in the Mongbwalu health zone.
- Activation of Shari Bridge PoC.

Indicators	Results
Proportion of activated PoCs	21.8% (12/55)
Number of travelers screened, % of	22,547
travelers screened, % of travelers	98.2%
refused screening, % of	2.0%
travelers who washed their hands, % of	97.4%
travelers who refused to wash their	2.6%
hands, % of travelers made aware of the risks	98.4%
Number of alerts notified;	3
% of alerts investigated in 24 hours	2 (66.6%)
Number of validated alerts	1
Number of alerts transferred to the CT/CTE or for EDS	1
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 109 samples collected and analyzed (33% positivity)
- **North Kivu:** 11 samples collected and analyzed, none of which came back positive for Ebola virus disease. However, 159 results remain pending due to a lack of reagents.
- **South Kivu:** 6 samples received and analyzed in the laboratory, all negative for Ebola virus disease.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Briefing on standard precautions, waste management and handwashing for 15 healthcare providers from 4 health centers (CS Aero, CM Chirurgial Croix Verte, CH Rwankole and CS Rwambuzi); briefing of 14 healthcare providers (4 CSR Nyangaray, 4 CSR Zengo, 3 CS Tsili and 3 CS Tchudja) on handwashing, biocleaning and waste management; decontamination of the Saint-Nicolas orphanage (AS Bankoko); opening of a ring around the Saint Nicolas orphanage in 3 health centers of AS Bankoko, 13 churches and 433 households.
- **North Kivu:** Scorecard evaluation in 5 health establishments in the Katwa health zone, the average IPC score in the assessed health establishments is 40%; briefing of 28 providers on the different IPC themes in the assessed health establishments; briefing of 5 healthcare providers in the Beni health zone; completion of 1 EDS in Katwa; decontamination of 2 health establishments and 1 household in the Katwa health zone; revitalization of IPC-WASH committees in five health facilities; reorganization of triage and isolation circuits in the Butsili and Ngongolio health centers.
- **South Kivu :** Evaluation of Scorecard in two structures in the Miti-Murhesa Health Zone (CH Lwiro: 42% and CS Kahungu: 35.5%) and development of improvement plans for these two health establishments; provision of IPC inputs: 100 handwashing kits, 40 of which were distributed to health establishments and 60 to the community, 40 decontamination kits deployed in the Miti Murhesa and Katana Health Zones.

4.5 Holistic care

Table 4. Patient movement within healthcare facilities as of June 5, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	236	8	7	251
Total admissions (24h)	37	1	0	38
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	0	7
Non-cases /	5	0	0	5
Recovered / Escaped (Suspects/Conf.)	10	0	0	10
Total discharges	22	0	0	22
of patients in isolation (End of	251	9	7	267
Day), including confirmed	83	5	1	89
(NC+AC) and suspected cases	168	4	6	178

Mental and psychosocial health activities

• Key figures

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
Proportion of newly confirmed cases that benefited from SMSPS interventions	9 (56.3%)	3 (50.0%)	0
Proportion of confirmed cases under follow-up that benefited from SMSPS interventions	14 (20.9%)	2 (28.6%)	3 (100.0%)
Proportion of new suspected cases that benefited from SMSPS interventions	12 (26.7%)	23	5 (100.0%)
Proportion of suspected cases under follow-up that benefited from SMSPS interventions	27 (22.0%)	87	8 (100.0%)
Proportion of laboratory results having been announced	36 (100%)	4	2 (100.0%)
Including positive ones	20 (55.6%)	4 (100.0%)	0
Proportion of children separated in daycare who benefited from SMSPS interventions	0	0	1 (100.0%)
Proportion of children separated in the community who benefited from SMSPS interventions	63	23	2 (33.3%)
Number of accompanying persons at the CTE who benefited from SMSPS interventions	37	10	12
Number of PPLs who benefited from SMSPS interventions	30	13	0
Number of households and communities that benefited from SMSPS interventions	52	67	0
Proportion of patients who received Dignity Kits at the CTE	0	0	0

4.6 Continuity of services.

- Preparatory meeting of ESS investigators from the Mongbwalu health zone.
- The evaluation of HGR Mongbwalu.
- Plan to improve the human resources situation (frontline service providers).

4.7 CREC

- **North Kivu:** 19,832 people reached; 3,221 households visited thanks to the involvement of 658 RECO; 813 community leaders engaged; 46 community alerts reported and 1 rumor clarified; 18 local radio stations engaged with 3 formats produced and broadcast.
- **South Kivu:** Mass awareness-raising of 405 final-year school students at the ENAFEP center in Kabunga on prevention measures and signs of Ebola virus disease; advocacy meeting in the village of Kahungu (Epicenter of Ebola) with 40 youth leaders, women leaders, school representatives, religious leaders on their involvement in the fight against Ebola; 6,961 people were sensitized and 1,567 households were also visited.
- **Ituri:** 45 economic operators (Federation of Enterprises of Congo (FEC) including 12 women sensitized on preventive measures of Ebola virus disease in their workplaces (shops, stores, markets, etc.) as well as in their living environments; start of sensitization in support of the reception and isolation of sick people at the Nyankunde Ebola Treatment Center (**Observation:** No reluctance of sick people to be isolated); sensitization on the prevention of Ebola virus disease for the benefit of FARDC soldiers in the operational sector in TSERE, Rwampara Health Zone, during the morning parade.

4.8 Logistics and Security

- Continued support for the pillars.

4.9. Security

- The security situation remains unstable in the health zones of Nizi and Bambu.

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the presence of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

5. PSEA (Prevention of Sexual Exploitation and Abuse)

- PSEA briefing of 41 (13 women) RECO from the Miti-Muresa Health Zone (Kahungu and Chaoboka Health Area) on EAS concepts, 495 555 (toll-free number). All signed the code of conduct.

5.2 MAIN CHALLENGES

Challenge No.	Impact	Action required
1. Reluctance to accept post-sampling mortem (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Acceleration of the construction of planned CTES
3. Weakness in contact tracing (overall rate 67.2%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of relays communities

4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6. Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action			Due date
		Responsible pillar	
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 41.2%) Surveillance		Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Catch-up plan for the 42 samples awaiting analysis in North Kivu	Laboratory	Day 2
9	North Kivu: discharge of the recovered case to Goma (Day 1), response to security incidents (ADF incursion), IPC training at the General Referral Hospital Virunga, CREC extension to 15,000 households		Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for IT/ECZS		Day 2

Activation of Shari Bridge PoC.



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